

# Mobilize & go-decision packet

Ambient Clinical Value Chain Activation

Tenant: Meridian Health System

Tenant key: meridian-health

Move ref: meridian:move:ambient-clinical-value-chain-activation

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The mobilization plan, adoption & change approach, the value-measurement model handed to Tower and the go-decision pack.

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# Mobilize & Handoff — adoption, measurement, go-decision

Ambient Clinical Value Chain Activation · meridian-health

Go-decision: NO GO

No-go: 2 Mobilize kill trigger(s) fired (measurement\_unwired, business\_case\_blocker\_open). The Move is not ready to leave Moves. See the fix-conditions for what must change before it is reconsidered.

## Mobilize readiness

|                       |                                          |
|-----------------------|------------------------------------------|
| Owned                 | Yes                                      |
| Adoptable             | Yes                                      |
| Measurable            | NO                                       |
| Operating-model owner | Dr. Morales (CMIO, Clinical Informatics) |
| Overall change load   | high                                     |

## Fired kill triggers

- measurement\_unwired: 1 committed value metric(s) cannot be wired to a recorded baseline: Cost-per-clinician-hour conversion coefficient. — fix: Revisit when every committed metric is wired to a Discover baseline value or an owned, dated plan to capture one.
- business\_case\_blocker\_open: The business-case critic has 1 open blocker(s): cfo\_monetisation\_blocked. — fix: Revisit when the business-case critic reports no open blocker.

## Conditions to clear

- measurement\_unwired: Revisit when every committed metric is wired to a Discover baseline value or an owned, dated plan to capture one.
- business\_case\_blocker\_open: Revisit when the business-case critic reports no open blocker.

## Adoption & change approach — seven dimensions

| Dimension        | Magnitude | Recommendation                                                                                                                                                                     |
|------------------|-----------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| impacted_roles   | high      | Three roles change; the clinician role changes most. Stage the rollout by specialty, leading with primary care where pajama time is highest.                                       |
| process_variance | high      | Specialty template governance varies documentation materially; the process-redesign workstream must land before the change rollout (it is budgeted in the Design & Plan estimate). |
| training_load    | moderate  | Role-based training: ambient-tooling curriculum for clinicians, a queue-triage curriculum for CDI specialists.                                                                     |
| incentive_change | moderate  | Re-point clinician scorecards away from raw note volume toward documentation quality and recovered capacity.                                                                       |
| manager_adoption | high      | Manager reinforcement is the highest-risk layer. A dedicated clinic-manager enablement track with explicit reinforcement in the hyper-care window.                                 |
| communications   | moderate  | Address the clinician narrative directly — ambient documentation recovers cognitive load and after-hours time; it is not a surveillance layer.                                     |
| hypercare        | moderate  | An 8-week hypercare window with elevated informatics support and a weekly documentation-quality review; exit when pajama time and CDI volume hold for two consecutive weeks.       |

## Impacted roles

| Role                      | Headcount               | What changes                                                                                                  |
|---------------------------|-------------------------|---------------------------------------------------------------------------------------------------------------|
| Primary-care clinician    | Not recorded — seed gap | Clinicians move from manual after-hours documentation to ambient-captured notes reviewed and signed in-visit. |
| CDI specialist            | 42                      | CDI specialists work an ambient-routed HCC query queue rather than a starved, manually-built one.             |
| Clinic operations manager | Not recorded — seed gap | Managers coach to the new documentation pattern and the recovered-capacity visit targets.                     |

## Value-measurement model — handed to Tower

| Metric                        | Baseline | Target | Owner        |
|-------------------------------|----------|--------|--------------|
| Primary-care pajama time      | 129      | 100    | CMIO         |
| CDI HCC-relevant query volume | 40       | 160    | CDI Director |

HCC capture on the MA panel

|                                                |                         |                         |                  |
|------------------------------------------------|-------------------------|-------------------------|------------------|
| 58                                             | 63                      | VP Revenue Cycle        |                  |
| Cost-per-clinician-hour conversion coefficient | Not recorded — seed gap | Not recorded — seed gap | VP Revenue Cycle |

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